

PACKAGE INSERT

1. NAME OF THE MEDICINAL PRODUCT

Albumin-GCC-JAKEL 20% Solution for Injection

2. QUANTITATIVE COMPOSITION

50 mL/vial contains,

Normal human serum albumin.....	10g
Sodium caprylate (as a stabilizer).....	0.13295g
N-Acetyl tryptophan (as a stabilizer).....	0.197g
Sodium chloride (as an isotonic aid).....	q.s.
Sodium hydroxide (as a buffer).....	q.s.
Water for injection.....	q.s.

Albumin-GCC-JAKEL 20% is a sterile aqueous solution containing 20% purified human serum albumin(weight/volume). Albumin-GCC-JAKEL 20% is prepared by the Cohn ethanol blood fractionation method from pooled human plasma with a poor salt content obtained from healthy blood donors.

The product is stabilized with Sodium caprylate and N-Acetyl tryptophan. Albumin-GCC-JAKEL 20% is osmotically equivalent to four times its volume of normal citrated plasma. A liter of Albumin-GCC-JAKEL 20% solution contains 113.5-138.7mM of sodium ion. The product contains no preservatives.

Albumin-GCC-JAKEL 20% is heated at 60°C for ten hours. This heat treatment completely destroys the causative agents of viral hepatitis(HBV, HCV) and HIV -1 and -2. This product is prepared from units of human plasma that have been tested and found nonreactive for HBsAg and antibodies to HIV -1 and -2 and HCV.

3. PHARMACEUTICAL FORM

Solution for injection

Albumin-GCC-JAKEL 20% is a yellowish or ambered, transparent liquid.

4. CLINICAL PARTICULARS

4.1 Therapeutic indications

- 1) Hypoalbuminemia caused by albumin deficiency(Burn, Nephrotic syndrome) and dysfunction of albumin synthesis
- 2) Hemorrhagic shock

4.2 Posology and method of administration

Albumin-GCC-JAKEL 20% Solution for Injection is administered intravenously.

Hypoalbuminemia caused by albumin deficiency (bum, Nephrotic Syndrome) and dysfunction of albumin synthesis. Inject intravenously.

The total dosage will vary with clinical condition and size of the individual.

- In adults

An initial dose of 100ml infusion is suggested.

Additional amount may be administered as clinically indicated.

- In children

A dose one quarter to one half the adult dose or dosage may be calculated on the basis of 1.25~2.5ml per kilogram of body weight.

Hemorrhagic Shock

In the treatment of the patient in shock with greatly reduced blood volume, Albumin-GCC-JAKEL 20% may be administered intravenously as rapidly as necessary in order to improve the clinical condition and restore normal blood volume. This may be repeated in 15-30minutes if the initial dose fails to prove adequate. In the patient with a slightly low or normal blood volume, the rate of administration is not more than 1 ml per minute. The usual rate of administration in children should be calculated on one quarter to one half the adult rate.

4.3 Contraindications

It is contraindicated in patients with severe anemia or cardiac failure in the presence of normal or increased intravascular volume, and with known hypersensitivity to commercially available albumin solutions.

4.4 Special warnings and precautions for use

Albumin-GCC-JAKEL 20% should be administered with caution to patients with low cardiac reserve.

Rapid infusion may cause vascular overload with resultant pulmonary edema.

Patients should be closely monitored for signs of increased venous pressure.

- Dehydrated patients may require additional fluids.
- Rapid administration to injured patients may result in vascular overload or bleeding from previously undetected sites.
- Use in pregnancy : Safety for pregnant women has not been established. The possibility of parvovirus B-19 infection cannot be excluded from the administration of Albumin-GCC-JAKEL 20%. In case of parvovirus B-19 infection, fetal disturbances (abortion, hydrops fetalis, fetal death) may occur. Albumin-GCC-JAKEL 20% should be given to a pregnant woman only if the expected benefit justifies the possible risk.

4.4.1 Special warnings

Although this blood product is made from plasma which has been screened or tested for some known transmissible agents including HIV(AIDS), hepatitis B and hepatitis C, the limited sensitivity and reliability of such tests and screening procedures and possible contamination with other or unknown agents must be borne in mind.

Chemical processing and virus inactivation stages included in the manufacture of this product are believed to render it safe from the risk of infection. Nevertheless this possibility must always be considered, and should be conveyed, whenever possible, to patients who may receive the product.

4.4.2 Special precautions

Spillage or Breakage

Do not use if a break of the container or spillage occur. Precautions should be taken to avoid contamination of cuts and abrasions, as well as to avoid inhalation or swallowing of the spillage.

Adequate disinfection can be obtained with the application of 1 % sodium hypochlorite solution for 15 minutes. Commercial bleaches may be diluted appropriately to obtain this concentration. Please contact the supplier if you have any problem.

4.5 Interaction with other medicinal products and other forms of interaction

Not applicable.

4.6 Pregnancy and lactation

There is very little experience of the use of albumin in the early stages of pregnancy.

4.7 Effects on ability to drive and use machines

Some of the effects mentioned under section "Undesirable Effects" may affect the ability to drive or use machines.

4.8 Undesirable Effects

Allergic or pyrogenic reactions are characterized primarily by fever and chills, rash, nausea, vomiting, tachycardia and hypotension have also been reported. Albumin-GCC-JAKEL 20% particularly if administered rapidly, may result in vascular overload with resultant pulmonary edema.

4.9 Overdose

Toxicity and treatment of overdosage : Administration of large quantities of albumin should be supplemented with red cell concentrates or replaced by whole blood to combat the relative anaemia which would follow such use.

If circulatory embarrassment or pulmonary edema should develop, the infusion must be stopped immediately and specific treatment given.

5 PHARMACOLOGICAL PROPERTIES

5.1 Pharmacodynamics

Action and Mode or Mechanism of Action :

The primary function of Albumin is that of providing colloidal osmotic pressure within the circulation. The intravenous administration of Albumin-GCC-JAKEL 20% results, therefore, in drawing extravascular fluid into the vascular system. Serum albumin which contributes to the buffer capacity of the plasma, has transport function. Although Albumin contains some amino acids, it provides only moderate nutritive effect.

5.2 Pharmacokinetic properties

Under normal conditions, the total exchangeable albumin pool is 4-5 g/kg body weight, of which 40-45 % is present intravascularly and 55-60 % in the extravascular space. Increased capillary permeability will alter albumin kinetics and abnormal distribution may occur in conditions such as severe burns or septic shock.

Under normal conditions, the average half-life of albumin is about 19 days. The balance between synthesis and breakdown is normally achieved by feedback regulation. Elimination is predominantly intracellular and due to lysosome proteases.

In healthy subjects, less than 10 % of infused albumin leaves the intravascular compartment during the first 2 hours following infusion. There is considerable individual variation in the effect on plasma volume. In some patients the plasma volume can remain increased for some hours. However, in critically ill patients, albumin can leak out of the vascular space in substantial amounts at an unpredictable rate.

5.3 Preclinical safety data

Human albumin is a normal constituent of the human plasma and acts like the physiological albumin.

In animals, single-dose toxicity testing is of little relevance and does not permit the evaluation of toxic or lethal doses or of a dose-effect relationship. Repeated-dose toxicity testing is impracticable due to the development of antibodies to heterologous protein in animal models.

To date, human albumin has not been reported to be associated with embryo-fetal toxicity, oncogenic or mutagenic potential.

No signs of acute toxicity have been described in animal models.

6 PHARMACEUTICAL PARTICULARS

6.3 List of Excipients

Sodium caprylate

N-Acetyl tryptophan

Sodium chloride

Sodium hydroxide

Water for injection

6.4 Incompatibilities

In the absence of compatibility studies, this medicinal product must not be mixed with other medicinal products.

6.5 Shelf-life

39 months from the date of manufacture.

6.6 Special precautions for storage

Store below 30°C. Do not freeze.

6.7 Nature and contents of container

50 mL/ vial (Type II treated soda-lime-silica glass vial, chlorobutyl rubber stopper and flip of cap.

6.8 Special precautions for disposal and other handling

Should not be used if they appear turbid or if there is sediment in the bottle , or more than 4 hours after the container has been penetrated. Any remaining contents should be discarded.

User Instruction:

Cleanse exposed rubber stopper with a suitable germicidal solution and prepare the administration equipment as follows:

- a) Close clamp on infusion set.
- b) With bottle upright, thrust piercing pin straight through stopper center. Do not twist or angle.
- c) Immediately invert bottle to automatically establish proper fluid level in drip chamber(half full).
- d) Attach infusion set, open clamp and solution to expel air from tubing and needle, then close clamp.
- e) Make venipuncture and adjust flow.
- f) Discard all administration equipment after use.

7 MANUFACTURER

GC Biopharma Corp.

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8 PRODUCT REGISTRATION HOLDER

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9. DATE OF REVISION OF THE TEXT

April 2025